

Stroke Code Activation & Interfacility Transfer Pathway

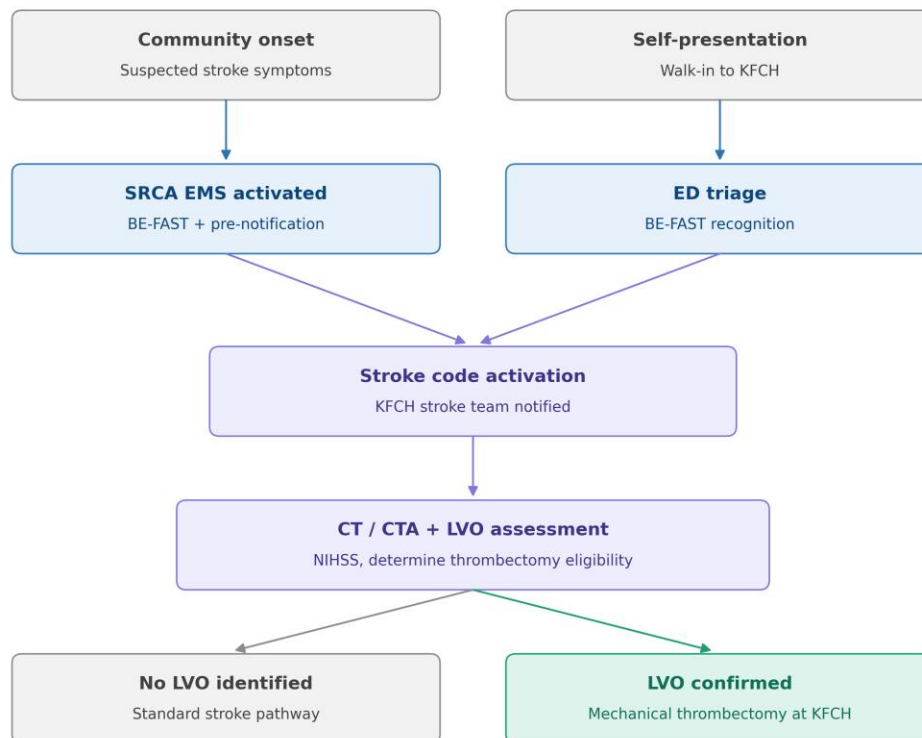
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King Fahad Central Hospital (KFCH) — Stroke Program

Alignment Note: This pathway is aligned with AHA/ASA Mission: Lifeline Stroke EMS pre-notification recommendations and Target: Stroke direct-routing principles, in support of Comprehensive / Thrombectomy-Capable Stroke Center certification requirements for interfacility transfer of large-vessel-occlusion (LVO) patients.

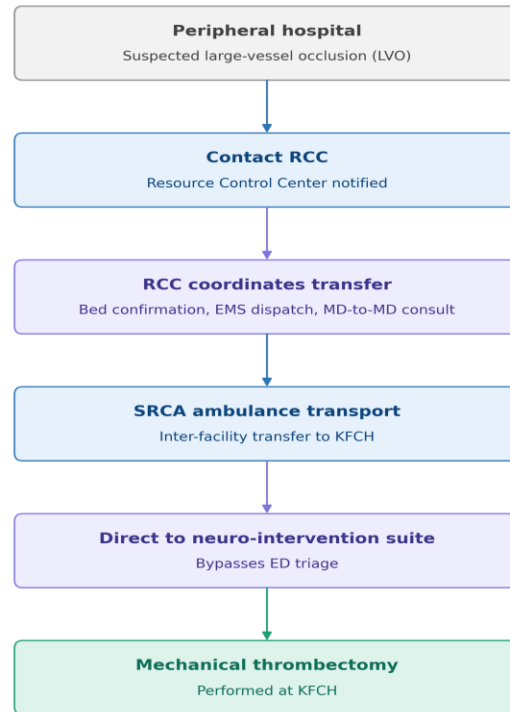
1. Stroke Code Activation Pathway

Two community-facing entry routes into the KFCH stroke code — pre-hospital activation via Saudi Red Crescent Authority (SRCA) EMS, and direct self-presentation through ED triage — converge into a single stroke code activation and imaging workflow.



2. Peripheral Hospital Transfer Pathway — Role of the Resource Control Center (RCC)

When a peripheral hospital identifies a suspected large-vessel-occlusion (LVO) stroke, the Resource Control Center (RCC) serves as the single coordination point that facilitates transfer to KFCH for mechanical thrombectomy, enabling direct routing to the neuro-intervention suite without a second ED evaluation.



3. RCC Coordination Detail

RCC function	What the RCC coordinates
Bed confirmation	Confirms real-time availability of the angio suite and post-procedure stroke unit / neuro-ICU bed at KFCH; identifies an alternate receiving point if the primary destination is unavailable; confirms accepting physician name.
EMS / SRCA dispatch	Requests an inter-facility SRCA transport unit specifying time-critical LVO acuity; confirms pickup location and departure time; relays LKW time, NIHSS, vitals, and current treatment to the transport crew; tracks transport in real time where available.
Physician-to-physician consult	Connects the sending physician directly with the KFCH neurointerventionalist or stroke neurologist on call to review imaging (occlusion site, ASPECTS), contraindications, and thrombolysis status, and to pre-authorize the treatment plan enabling ED bypass.
Transfer log & escalation	Maintains a single timestamped record (call received, bed confirmed, EMS dispatched, departure, KFCH arrival) feeding door-to-groin and transfer-time KPIs; escalates to the Stroke Program Coordinator or Medical Director if any leg stalls.

Prepared for: Stroke Program Governance — Accreditation Reference